

PulseSync Medical - Cardiac Device Services

Patient: Mark Smith	Patient ID: PSM491002	Date of Procedure: 21-06-2025
Document: Pacemaker Implant Report		

Procedure Details

Procedure	Dual-chamber pacemaker implantation (right pectoral)
Indication	Symptomatic high-grade AV block with bradycardia and pauses
Anesthesia	Conscious sedation; local anesthetic at incision site
Antibiotic Prophylaxis	IV cefazolin prior to incision; single peri-operative dose
Venous Access	Right subclavian/axillary vein under fluoroscopic guidance
Fluoroscopy	Used for lead placement and final position confirmation

Device & Leads

Generator	PulseSync DuoRate DR-220; Serial: PS-DR220-00123
Atrial Lead	Active-fixation 52 cm; final position: right atrial appendage
Ventricular Lead	Active-fixation 58 cm; final position: right ventricular septum

Intraoperative Measurements (Final)

Parameter	Atrial	Ventricular
Sensing (intrinsic)	P-wave 3.4 mV	R-wave 10.2 mV
Capture Threshold	0.5 V @ 0.4 ms	0.7 V @ 0.4 ms
Impedance	540 Ω	590 Ω

Programming at Closure / Discharge

Mode	DDD 60-130 bpm
AV Delays	PAV 180 ms / SAV 150 ms
Outputs	A: 2.5 V @ 0.4 ms; V: 3.0 V @ 0.4 ms
Sensitivity	A: 0.5 mV; V: 2.0 mV

Intraoperative Course & Imaging

Course	No complications. Hemostasis achieved. Pocket irrigated; generator anchored; layered closure.
Fluoroscopy/CXR	No pneumothorax. Leads in expected positions; appropriate slack.

Post-Procedure Plan

Medications	Complete antibiotic prophylaxis as ordered; resume home meds unless instructed otherwise.
Wound Care	Keep dressing dry for 48 hours; avoid submersion; watch for redness, swelling, drainage.
Activity	Limit ipsilateral arm elevation above shoulder for 1-2 weeks; avoid heavy lifting >5 kg for 2 weeks.
Follow-up	Wound check in 7-10 days; device interrogation at 4-6 weeks; enroll in remote monitoring.
When to Seek Care	Syncope, presyncope, palpitations, fever, increasing pain, or any wound concerns.